SYNTHETIC TEST DOCUMENT · FICTIONAL PATIENT · NOT A REAL MEDICAL RECORD

Southside Oncology (synthetic)

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14 October 2026

Dr Morgan Grey (synthetic)
Example Family Practice
22 Sample Pde, Hurstville NSW 2220

Dear Morgan,

Re: Dana EXAMPLE, DOB 14/05/1962, 9 Placeholder St, Hurstville NSW 2220

Diagnosis: Stage IV EGFR-mutant (exon 21 L858R) adenocarcinoma of the right upper lobe, diagnosed January 2024 (TTF-1+, Napsin A+). Initial staging showed right pleural disease and rib/T12 bone metastases.

Past history:
- Left breast cancer 2017: invasive ductal carcinoma, T1N0, ER/PR positive, HER2 negative. Wide local excision and sentinel node biopsy (March 2017), adjuvant radiotherapy, and letrozole for 5 years (completed April 2022). No evidence of recurrence.
- Hypothyroidism.

Treatment to date:
1. Osimertinib 80 mg daily, February 2024 to June 2025: partial response, then progression (chest wall and pleura).
2. Carboplatin and pemetrexed, July 2025 to December 2025 (6 cycles, then pemetrexed maintenance stopped at progression in December 2025).
3. Docetaxel 75 mg/m2 three-weekly, January 2026 to present; last cycle 18 September 2026.

Medications: Docetaxel as above; levothyroxine 100 mcg daily; dexamethasone 8 mg on the day before, of and after each docetaxel cycle only (premedication); ondansetron 8 mg prn.

I reviewed Dana today with the restaging CT of 2 October and the MRI brain of 12 October. The CT shows progressive disease: the right chest wall nodule has grown from 13 to 17 mm and the loculated right effusion is slightly larger. Her last pleural tap was in March 2026 and she is not breathless at rest. The MRI shows a new solitary 6 mm left frontal metastasis. She has no headaches, focal weakness or seizures and is not on regular steroids.

She is managing at home and walks daily, although she tires easily after docetaxel. Haemoglobin is 104 g/L; renal and liver function are normal. An echocardiogram in August 2026 showed an LVEF of 60%.

We discussed that docetaxel is no longer controlling her disease. Archival tissue from January 2024 (MP-24-000731) is available; a repeat biopsy of the chest wall nodule could clarify the resistance mechanism, and she would accept this.

Plan:
1. Stop docetaxel.
2. Discuss at the thoracic MDT on 21 October regarding stereotactic radiosurgery to the brain lesion versus surveillance MRI in 4 weeks.
3. Explore clinical trial options for EGFR-mutant NSCLC after osimertinib, including antibody-drug conjugates.

4. Consider core biopsy of the chest wall nodule for tissue NGS including MET amplification.
5. Review in 2 weeks.

Kind regards,

Dr Alex Rivera (synthetic)
Medical Oncologist